



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : جليله علي مهني فارس
المحافظة : القاهرة
الحى / المدينة : عين شمس
العنوان : --
رقم البطاقة : 25809022100503 رقم التليفون : 01223841212
تاريخ تسجيل الإستمارة : 30-09-2026
العمر (تاريخ الميلاد) : 02-09-1958 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة
قسم العلاج :
الأورام [11129]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /----- /د. /----- /د. /----- /د. /-----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: جليله علي مهني فارس National ID: 25809022100503	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



PMS

معه ذري

مقلاعية



MDT FORM

Name: علي . فارس .	MRN: 11173
Age: 68 Y	ID:
Menopausal Sta: POST	Phone: 01228773623
NRS SCORE:	Marital Status: WIDOW
BMI:	Family History:
CO Morbidity: HTN, CARDIAC (PCI ON ASPIRIN)	Reason Of MDT: NACT

Clinical Data:

BX: IDC

ER: 0/8 PR: 3/8 HER2: +3 KI67: 20%

R

L

Sonomammography:

PATHOLOGICALLY PROVEN UPPER CENTRAL MASS 3.2*2.3 CM WITH PICTURE IMPRESSIVE OF PAGET,S DISEASE

Mets Workup:

History:

ACASE OF RT BREAST CANCER DIAGNOSED IN 9/2025 AND WAS PLANNED TO START TCHP BUT LOST FOLLOW UP TILL 7/2026

NEW ASSESSMENT : CT CAP WITH CONTRAST

RT BREAST UIQ MASS 2.5*2.4 CM

FEW RT AXILLARY LNS LARGEST 10*9 MM

RT LUNG SUBCENTIMETRIC NODULES ? METS ???

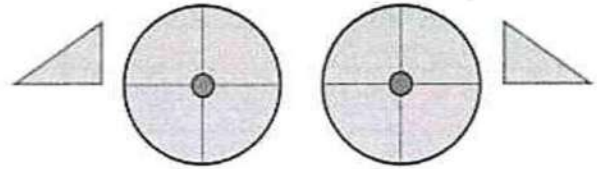
BONE SCAN : FREE

Recommendations:

MDT Outcome & Plan

radiology review : nodules not metastatic (free of metastatic)

for TCHP x 6



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. حنان عزت	د. احمد نجيب				

أحمد

Patient Name : الأستاذ / جليله علي مهني فارس
 Patient No. : 25809022100503
 Path No. : 1329/25
 Sex : Female
 Age : 66 Y
 Referring Doc :
 Request No. : 13007229
 Request Date : 15/04/2025
 Verification Date : 05/05/2025
 Financial Category : اورام دار السلام - نفقة الدولة
 Department : Out Patient
 Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast mass (BIRADS 4c).
 Nature of Specimen: Tru-cut biopsy, collected at 14/04/2025.

Gross Examination:

- Multiple grayish white tissue cores up to 1.5 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Widely sclerotic and necrotized cores of Invasive duct carcinoma, Grade II.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Not detected.
- * Perineural invasion: Not detected.
- * Angioinvasion: Not detected.
- * Microcalcifications: Not detected.
- * IPT (Positive control showed appropriate reactivity):
- ER: No nuclear reaction of tumor cells (score 0/8).
- PR: Weak nuclear reaction in 10% of tumor cells (score 3/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 20% nuclear reaction of tumor cells.

برأى
 د. محمد
 د. محمد

Diagnosis:

Right breast mass, Tru-cut biopsy:
 Widely sclerotic and necrotized cores of Invasive duct carcinoma, Grade II.
 PT:
 ER: Negative (score 0/8).

Verified by :

Dr.Toka Medhat

Printing Date : 25/05/2025 23:59:06

Page 1 of 2

Printing By : hermai



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - دور نيش النيل - مصر القديمة - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name : الأسماءه / جليله علي مهني فارس :	Request No. : 13007229
Patient No. : 25809022100503	Request Date : 15/04/2025
Path No. : 1329/25	Verification Date : 05/05/2025
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 66 Y	Department : Out Patient
Referreing Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

- PR: Positive (score 3/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 20% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr. Walaa Ghaoniam

Dr. Yasmine Essam

Verified by :

Dr.Toka Medhat



Patient's Name: **Jalilah Ali Mahani Faris**
 Date: **2026-09-02**
 Referred by: **Prof.Dr. no doctor**
 Hospital: **Goustave Roussey International Egypt**



Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Pathologically proven right breast Her 2 enriched carcinoma, re-evaluation study.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- Scattered Rod like benign macro-calcifications.
- Upper central middle third irregular speculated high density mass lesion with surrounding distortion and overlying as well as central breast skin thickening.
- **Ultrasound** demonstrated 12-1 o'clock, middle third, 3 cm from nipple, reaching the skin pathologically proven malignant mass lesion measuring 32 x 23 mm with mild internal vascularity on color Doppler study.
- Thickened skin reaching 5 mm with nipple/areola destruction.
- Non-specific axillary LNs. No pathological infra or supra-clavicular lymph nodes.

Left breast.

- Scattered fibroglandular densities. (ACR b)
- Scattered Rod like benign macro-calcifications.
- No speculations, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 6): Pathologically proven upper central malignant mass lesion with picture impressive of Paget's disease. Benign calcifications impressive of of plasma cell mastitis.

Left breast (BIRADS 2): Benign calcifications impressive of plasma cell mastitis.

Best Regards,

DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

<u>Category & recommendations</u>	<u>Suspected Cancer Risk</u>
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	
	98-100% BIRADS 6: Pathologically proven malignant

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MDT FORM

١٨/٩ / 2025 G

Name	حليته علي موني	MRN	١٥٤٤
Age	67 Y	ID	٤٥
Menopausal status	post-menopausal	Phone	٠١٠٠٨٨٢٨٦٣٥
NRS SCORE		Marital status	١١٢٢٢٢٢٢٢٢٢٢
BMI		Family history	
CO morbidity	HTN PC I - on Aspirin	REASON OF MDT	٢٠٨.٩.٢٢١.٠٠١

Clinical Data

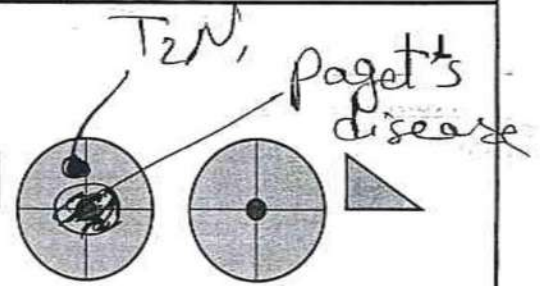
BX: Invasive duct carcinoma

ER: 0/8 PR: 3/8 HER 2: +3 Ki67: 20%

Sonomammography: Rt breast: upper
central suspicious lesion

Mets Workup:

CAP: few bilateral pulmonary nodules
of indeterminate nature
Bone Scan: free



Recommendations:

د. ابراهيم صفي الحبال
٢٠٨/٩/٢٥

MDT OUTCOME & PLAN

MDT TCHD x 18 week

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. محمد حسن	د. محمد حسن	د. محمد حسن			

Name : Jalilah Ali Mohany Faris

Date : 02-07-2026 09:01

Branch : Dokki

Medical No. : 407619

Ref.Physician Prof : Goustav Roussey Egypt

Whole-Body Bone Scintigraphy Report

Indication: Evaluation and follow-up of a primary left breast tumor (not surgically resected) presenting with bone aches. This is the patient's first bone scan.

Technical Procedure: Following I.V administration of Tc-99m MDP, static whole-body anterior and posterior skeletal images were acquired.

Findings:

- The scan demonstrates a normal, physiological distribution of the radiotracer throughout the axial and appendicular skeleton. Both kidneys and the urinary bladder are normally visualized with expected tracer excretion.
- There is no definitive scintigraphic evidence of focal, intensely abnormal osteoblastic activity or asymmetric "hot spots" to suggest focal secondary osteoblastic metastatic disease in the visualized skeleton.
- Mild, symmetrically increased tracer uptake is noted involving both shoulder joints, both knee joints, and along the lower lumbar spine. These patterns are characteristic of bilateral degenerative joint disease and prominent osteoarthritic changes, correlating with the patient's age and clinical presentation of generalized bony aches.

Conclusion:

- Negative bone scan for clear osteoblastic metastatic disease.
- Scintigraphic evidence of generalized degenerative osteoarthritic changes, most pronounced in the shoulders, knees, and lower spine.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

ECHO REPORT

NAME : جليله على مهنى DATE: 11 / 11 / 2025
ID NUMBER : ١٠٤٩ / ٩٥ SEX : f AGE: ٤٧

EF	<u>65.1</u>	55-75%	PWT	<u>1.0</u>	0.6-1.1cm
LVEDD	<u>4.1</u>	3.5-5.8cm	AO.R	<u>2.7</u>	3-3.7cm
LVESD		2-4cm	LA	<u>3.9</u>	2-4cm
FS		25-45%	RV		0.7-2.3com(supine)
SWT	<u>1.1</u>	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions normal
LV Wall Thickness normal
LV Diastolic Function DD grade II
SWMA swma
LA and Ao Root Dimensions normal
Mitral Valve Mild MR
Aortic Valve Mild AR
Tricuspid Valve Mild TR
Pulmonary Valve normal
Right Side and Systolic Function normal
Intracardiac Masses and Effusion normal

DR.
Olga